National Cancer Control Programme (NCCP) 00319 · clinician draft

Carboplatin + Oral Etoposide

Which treatment this guide covers This guide covers carboplatin plus oral etoposide for extensive-disease small-cell lung cancer in people unsuitable for cisplatin-based treatment. It does not cover an intravenous etoposide schedule or an added immunotherapy medicine. If the prescription differs, ask for its matching guide and consent discussion.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 1 · carboplatin AUC5 plus oral etoposide

Active medicines: carboplatin + etoposide

Emetogenic risk: high

Medicine	Dose / route	When
Aprepitant	125 mg orally Day 1; 80 mg orally Days 2–3	Before systemic anticancer therapy (SACT) on Day 1, then daily on Days 2–3
Ondansetron	16 mg orally	Day 1 Adjust for QT/individual risk.
Dexamethasone	12 mg orally Day 1; 8 mg orally daily Days 2–4	Dedicated carboplatin AUC ≥4 NCIS V8 row This is the dedicated carboplatin row. Reconcile with the full multi-day plan; no duplicate steroid courses.
Metoclopramide	10 mg orally three times daily as prescribed	Subsequent days NCIS row does not specify an end day or PRN qualifier; prescriber/pharmacy must specify duration and suitability.

Additional phase-specific support

- Dedicated carboplatinAUC≥4NCISV8row; routine olanzapine is not included in this row.
- Only Day 1contains high-risk carboplatin. Days 2–3oraletop are minimal–low risk and need their own daily review within one reconciled plan.
- NCIS multi-day principle:appropriate prevention on each treatment day and two days afterwards. Define the Day 4–5/rescue plan without inventing extension of every high-risk medicine.
- Metoclopramide row does not define end day or PRN qualifier; prescriber/pharmacy must set duration and suitability.
- Check QT risk for ondansetron and all interactions; local policy and individual prescriptions apply.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

National Cancer Control Programme (NCCP) 00319 v5 · pNCCP 00319pp1,4–5;NCISV8Table1p4andmulti-daynotep5 · NCCP NCIS Medical Oncology antiemetics V8, Table 1

Sources checked 2026-11-02 · SACTCheck v0.84.29 · Regimen-level drafts; prescriber and oncology-pharmacy verification required

Days 2–3 · oral etoposide, then two-day post-treatment support

Active medicines: etoposide

Emetogenic risk: minimal to low

Medicine	Dose / route	When
Continue the reconciled Day 1 prevention plan	As individually prescribed	Includes the Day 2–3aprepitant and Day 2–4dexamethasone from the carboplatin reference row where prescribed Do not repeat Day 1doses or add a second high-risk course.
Oral-etoposide-day prevention / rescue	Individualised to daily risk, symptoms and existing medicines	Before oral treatment on Days 2–3 and appropriate support for two days after finalSACT (Days 4–5) National Cancer Control Programme (NCCP) 00319 calls oral etoposide minimal–low. NCISV8p5provides a daily-risk principle, not an explicit universal Day 5drug/dose; pharmacy must reconcile it.

Additional phase-specific support

- Prescribe one clear multi-day schedule with stop days.
- No automatic routine olanzapine, prolonged aprepitant, or copied five-day cisplatin schedule.

- Patient records exact oral doses and contacts team about missed/vomited doses.
- Regular premedication is not usually required unless previous hypersensitivity; no additional routine supportive medicines specified by00319.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00319 v5 · pNCCP 00319pp1,4–5;NCISV8pp4–5 · NCCP NCIS Medical Oncology V8: multi-day principle

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